

Literature Review Assignment

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I have no known conflict of interest to disclose.

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Abstract

This paper is a literature review of the research to date on how problem pornography use causes emotional dysregulation. The literature comes from peer-reviewed journal articles and books with primary source research studies. Much current research points to other causes of emotional dysregulation associated with problem pornography use other than shame and addiction in the clinical sense. Reading and reflecting on as much of the current literature as possible has led to the conclusion that moral incongruity, shame, and one's view of God are all variables regulating the degree to which individuals are emotionally dysregulated when using pornography. However, moral incongruity is the present-day explanation for distress related to pornography use.

Keywords: moral incongruity, sexual shame, God image, depression

Literature Review

Pornography is a mode of fantasy and arousal used by many regardless of how it affects their relationships and individual emotions. There is no shortage of research designed to study pornography and why pleasure is so powerful. The research into the effects of pornography on relationships and individual emotional regulation leads many researchers to agree that pornography use can be harmful. However, the types of deleterious effects on users' psyche, neurobiology, and relationships are up for debate. Within this debate, there is a recent question as to whether the medical model of addiction best conceptualizes problematic pornography use. Volk et al. (2016) share the emerging view that identifying as a sex addict may cause more harm than effect healing. In a 2015 study, Grubbs et al. pointed to newer understandings of causes of (i.e., mediating and moderating factors) emotional dysregulation and relationship issues associated with pornography use. The most recent discussions conclude that other factors exist to explain problematic pornography use other than a medical model of addiction.

Moral Incongruence and Pornography Use

One of the factors proposed to contribute to individuals seeking out pornography even when it is problematic and causes emotional dysregulation is morality (Duffy et al., 2016). Specifically, moral incongruence (Grubbs, 2019). Moral incongruence due to pornography use is the leading explanation of dysregulation, more so than pornography use itself (Grubbs et al., 2019). Using pornography for pleasure when it is believed by the user to be immoral, especially when one's religious views point to immorality, increases the shame and emotional dysregulation related to use. Research like Grubbs et al. (2019) discussed pornography-related problems arising from moral incongruence (PPMI) as broadly related to depression. One 2016 study with participants in convenience samples across a qualitative study resulted in participants

experiencing greater shame over hypersexual behavior when their religious views called for this behavior to be a severely immoral act (Karaga et al.). While previous research focused on shame as a mediating factor and a generalizing view of how early attachment bonds caused addiction (Adams & Robinson, 2001), research in pornography use in the last five years has taken a turn toward individual use and how religion and morality effect perceived problems and addictions (Floyd & Grubbs, 2022). Shame tendency (developed by early attachment styles) is a moderating factor for emotional dysregulation, including depression, rather than the cause of emotional dysregulation. The cause for emotional dysregulation connected to pornography use is increasingly seen as moral incongruence.

Moral incongruence can be defined as believing that a certain behavior or character trait is right or wrong and allowing oneself to partake in said behavior or not partake in said behavior or misalign one's character with beliefs of wrongness or rightness. Others have simply stated this as the degree to which one's behavior matches one's beliefs (Floyd & Grubbs, 2022). Given this definition, moral incongruence should be measured by at least two scales: one to measure the degree of belief and a second to measure the degree of incongruent behavior. For moral incongruence related to pornography use, The Cyber Pornography Use Inventory (CPUI) could measure participants' beliefs about pornography (Volk et al., 2019). The CPUI is a 40-item self-report with six subscales. It is a Likert-type report from strongly agree to strongly disagree (Grubs et al., 2010). The CPUI was designed to target online pornography use with allowances for religious moderation. Historically this measure has been proven in multiple studies (Grubs et al., 2010). The CPUI in multiple forms was shown by Floyd et al. (2022) to be a valid measure. This measure could be used in conjunction with Grubs et al. (2015) assessment designed specifically for a particular study to measure the amount of moral disapproval of pornography

use. This measure asks four questions with answers on a Likert-type scale from 1 (not at all) to 7 (extremely). The study this measure was designed for yielded a Cronbach's alpha of above .70 in all but a few subscales. Together, these two measures would give a clearer understanding of the degree to which individuals use pornography in a morally incongruent manner with beliefs or PPMI. When a clearer picture of PPMI is attained in an individual case, the degree to which shame and other factors mediate and moderate emotional dysregulation can be evaluated.

Sexual Shame

Shame is an emotion related to the concept of the self. It can help make decisions or be harmful when linked to numerous psychological disorders. When shame is connected to problematic pornography use, it is often seen as harmful and leads to depression and other distressful emotions. Reid et al. (2009) found that individuals with hypersexuality disorder use withdrawal and negative self-criticism to counteract shame. Carnes et al. (2004) focus on a part of this shame cycle in the *Handbook of Addictive Disorders*. They relate much of what Carnes and others using a medical model of addiction pointed to in early understandings of problematic pornography use. If not handled well by therapists and clients, that shame can be a precipitating factor in users returning to problem pornography rather than staying with healthy habits (Carnes et al., 2004). Shame increases the frequency of the use of pornography. Still, more than just shame mitigation for the reduction of pornography use, clinicians should also consider how users cope with shame in other ways. According to Volk et al. (2019), these coping mechanisms (especially shifting responsibility to others) contribute significantly to decreased therapeutic outcomes. These and other studies show evidence that shame and behaviors that cause dysregulated emotions are related. For example, shame was proven to be a positive predictor of hypersexual behavior in a study of pornography users already in treatment for hypersexuality

(Gilliland et al., 2011). However, the most recent evidence points to shame as an outcome of problem behavior or as a mediator of the outcome of problem behavior rather than as the cause of problem behavior (F. Volk, personal communication, October 13, 2022). As shame is considered part of the dysregulation of emotions in connection to problem pornography use, one must account for the degree to which shame is precipitating or mediating distressful emotions. To do so, the measurement of shame should be a part of the overall approach for each person seeking to regulate emotions.

Shame can be measured using self-consciousness scales. Lear et al. (2022) identified and rated nineteen shame assessments. The Test of Self-Conscious Affect (TOSCA) has a history of validity and rigorous study according to this same study. Correspondingly, they found that with version 3 (TOSCA-3), Cronbach's α is between .75 and .82 for structural validity. The 16-item self-report shows internal validity and construct validity as well ($\alpha = .91$). The above study also reported that a qualitative meta-analysis of the TOSCA-3 showed initial validity, but they indicated a lack of peer-reviewed research statistics. Overall, the study suggests that the TOSCA and other general shame measures may be too generalized. Further research on measures that take a case-by-case individual assessment of shame may need further research. Lear et al. believe that shame measures such as the TOSCA may combine the self-consciousness of shame with the behaviors associated with shame. These findings do not negate the helpfulness of the TOSCA assessment. The measure is still helpful for collecting evidence for one's view of self, including how one operates out of a negative self-image. This is important as the literature shows that when a negative self-image is correlated with the above-discussed moral incongruence, the result is distressful or dysregulated emotions. Shame is empirically shown to affect how one feels due to problematic pornography use. However, shame is not the only

reason individuals dysregulate when using pornography. There is also evidence for one more moderating factor of one's view of God.

God Image

God image can be divided into two basic categories: I see God as cruel, or I see God as close. Whether an individual sees God as cruel or close is determined less by psychological constructs and more by experience with representations of God in one's life (Lawrence, 1997). For example, one has experiences with one's biological father that relate to the concept of God as a person (Lawrence). An emotional wound or broken bond perceived to be caused by a biological father could easily be projected onto God as a Father wound. In the study referenced above by Lawrence, the prediction is made that individuals with low self-esteem would have a negative or wounded view of God. This is in keeping with the current understanding of how problem pornography causes emotional dysregulation. That is, shame and religion do not cause emotional dysregulation, but rather shame and one's view of God mediate and moderate emotional dysregulation when problem pornography use is incongruent with one's moral views. Although increased shame and guilt according to a quantitative research project by Murray et al. (2007), increased shame and guilt can be highly correlated with a lack of perceived relationship with God. However, the study found that shame was less correlated with sexuality. By the author's admission, this is not in keeping with other contemporary research, and the disparity was attributed to the lack of significant shame measures. The study indicated the trend in current literature's finding that religion or God view are not causes for emotional dysregulation. In keeping with Grubbs et al. from 2015 and Floyd and Grubbs from 2022, shame increases as one's view of self becomes negative, which is partly caused by religious views and an individual's God view. God view is an important variable in understanding how moral

incongruity causes depression and other negative emotions for this reason. Thankfully the field has measures to allow individuals to understand how they view God, cruel or close and how these differing views mediate depression.

The God 10 measures exactly this; God as cruel, distant, and loving. The God 10 is a questionnaire answered in a Likert-style fashion from not at all to extremely. The format follows a prompt about God (i.e., Generally speaking, I Imagine God as being...) with 10 adjectives that follow. Allowing practitioners to measure an individual's view of God is also seen as the degree to which one has divine struggles or if an individual wrestles with feeling loved by a higher power. The God 10 measure correlates negative views of God well with divine struggles (Exline et al., 2015), further understanding how moral incongruity affects emotional regulation.

Emotional regulation in context with moral incongruity can be measured by other assessments as well. Lawrence (1997) found the God Image Scale (GIS) to be a psychometrically reliable instrument for measuring how an individual sees God. Lawrence hypothesized that the GIS correlates self-esteem with one's image of God. Tisdale et al. (1997) state that the GIS has subscales to measure what one believes about whether God is there for them and wants them to grow and one's experience of the goodness of God. Compared to the original God Image Inventory (GII), the shorter GIS is a psychometrically valid measure (Tisdale et al., 1997).

Using these measures to assist individuals in determining their view of God helps them understand why their view of self would be negative, why moral incongruity exists, and why they are willing to seek pleasure through problem-causing pornography use. The result of emotional dysregulation, including depression, is treatable when seen through this construct. Before moving on to research gaps on this topic, the definition of the final variable of depression needs to be considered.

Depression

The *Diagnostic and Statistical Manual of Mental Disorders* (American Psychological Association, 2013) defines major depressive disorder with a specific criterion. As well as subjective feelings of distress, one must report physical manifestations. Depressive symptoms include a lack of motivation for everyday tasks, diminished pleasure for most activities, changes in appetite, changes in sleeping patterns, inability to concentrate on work or school, and fatigue. The etiology of major depressive disorder can be multitudinous and all factors such as grief as a complicating factor or addictions as comorbid disorders should be considered. For this paper, clinically significant distress, which leads individuals to seek help, in connection to problematic pornography use was the focus of the literature search. With this criterion in mind, Droubay et al. (2020) found evidence that when pornography use is viewed as morally wrong, shame is highly correlated to general distress. Their study showed that moral incongruence, shame proneness, and emotional dysregulation correlated with subjective distress. Past studies, such as Wright et al. (1989), indicated that shame, moderated by poor self-image, causes depression. Additionally, where many studies have focused on depression and/or distress and their connection to internet sex, Volk et al. (2019) point out that depressive symptoms often influence self-reports of life satisfaction. Both above studies connect a negative view of self (shame) with distress over pornography use. Overarchingly, the evidence is beginning to correlate moral incongruence with psychological distress/depression (Droubay et al., 2020; Floyd & Grubbs, 2022; Grubbs et al., 2010, 2015, 2019, 2022; Volk et al., 2016, 2019; Wright et al., 1989) when problem pornography use is seen as morally wrong. However, these studies, and others, indicate ways in which future research could contribute to our understanding.

Current Study

The literature search on these topics yielded much helpful data for research and clinical applications. A few areas offer an opportunity for further research, however. For example, more research is needed with treatment modalities focusing on moral incongruent related distress versus a broad general category of dysregulation due to pornography use (Floyd & Grubbs, 2022). Subjects who do not report a religious worldview or a moral theory based on traditional views of God could be important in this area. Much of the literature is homogenized around religious individuals. Non-religious individuals who report moral incongruence could offer insight into what defines their self-worth and, thereby, where shame adds distress.

In addition, Grubbs et al. (2022) state that more research is needed in cultural contexts where moral incongruity (MI) is believed to create distress and self-defined addictions. Determining which contexts contribute to a greater or lesser subjective moral incongruence could help distinguish further between religion and God view as moderating variables or other unidentified variables such as cultural values and expectations. This suggestion keeps MI as the output responsible for distress but helps clinicians understand where to intervene for a more successful outcome. However, many still disagree over where to intervene in problem pornography use that causes distress.

The question of how shame affects hypersexuality needs more attention and clarification among clinicians (Reid et al., 2009). Additionally, further research is needed to test models concerning men and hypersexuality related to shame proneness to determine an empirical treatment (Reid et al., 2011). Shame is still a variable in distressful emotions connected to problem pornography use. The hypothesis could be that addressing the cycle of hypersexuality at shame, regardless of MI, is still a valid intervention. Yet, clarifying this interaction further

could produce more successful interventions. Addressing moral incongruence, shame, the God view, and depression concerning problematic pornography use with various counseling models have yielded positive outcomes. Chisholm and Gall (YEAR) suggest; however that future research opportunities lie in determining how different spiritually focused mindfulness-based treatments impact shame in the role of pornography addiction. Their research suggests that this model of counseling theory is effective, but its effectiveness in differing populations is unknown. As well as clinical implications and moral incongruence research, the hypothesis of non-medical understanding of problem-pornography use is still under consideration.

In the area of addiction identification being harmful or helpful, future research is needed to understand how externalization influences pornography users (Volk et al., 2019). The above study saw evidence that blaming others for incongruent behaviors was a precipitating factor in feeling depressed. This is an important factor in understanding and treating problematic pornography use. However, broadening the scope of the study to include more diverse populations and populations who view pornography use differently is a suggestion by the authors for a more generalizable study outcome.

In all, broadening the research to include populations in other countries and marginalized cultures could prove beneficial. Also, distinguishing between male and female responses to moral incongruity and shame would provide additional evidence for clinical support of each gender. As research in this area adapts to the findings that moral incongruence is highly correlated to psychological distress, clinical applications should be sought to help clients behave in the way they believe.

Conclusion

As simplistic as the idea of behaving the way one believes is, this ability is elusive to many when it comes to pushing away from pornography use. The ubiquitous nature of internet access, the pleasure derived from neurochemicals released during orgasm, and the self-deprecating shame built-in users during emotional development keep them returning to problem pornography use despite the distress it causes. Early understandings of this phenomenon centered on a medical model of addiction and sought to treat individuals like other addictions. Where these early researchers, such as Carnes, made headway in the field, the current trend in seeking to understand shame and the concept of self in relation to moral incongruence is on the rise. Shame as a concept of self and religion and view of God are evidenced to play a role in distress caused by pornography use. Yet, as more research in moral incongruity is undertaken, there is more evidence for moral incongruence as the cause for distress than pornography use itself, shame, or God view alone.

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